Metro Health Medical Center - Acute Care Pavilion

CLINICAL DOCUMENTATION — NARRATIVE SURGICAL DISCHARGE SUMMARY V1

Patient: Susan Jones | **MRN:** PAT-60014 | **DOB:** 1973-04-16 | **Gender:** Female | **Doc ID:** NARRATIVE_0014
Encounter Date: 2025-05-16 | **Discharge/Note Date:** 2025-05-18 | **Provider:** Dr. Gregory House, MD

PREOPERATIVE DIAGNOSIS & PROCEDURE

The patient underwent uncomplicated laparoscopic cholecystectomy for symptomatic cholelithiasis under general endotracheal anesthesia.

HISTORY OF PRESENT ILLNESS

The patient is an adult who presented electively for surgical intervention following conservative management failure. Pre-admission testing confirmed that preoperative platelet count was 245.5 10*3/uL. Preoperative metabolic screening showed a baseline creatinine of 0.09 mg/dL (LOW). The patient was cleared from a cardiopulmonary standpoint and taken to the operative suite.

POSTOPERATIVE HOSPITAL COURSE

The patient was extubated in the operating room and transferred to the post-anesthesia care unit in stable condition. Postoperative recovery was uneventful. For pain and infection prophylaxis, the patient was prescribed Amlodipine 2.5 mg orally every morning. On postoperative day one, postoperative glucose remained stable at 87.6 mg/dL. Surgical incisions were clean, intact, and without erythema or purulent drainage.

POST-DISCHARGE MEDICATIONS

Discharge medications: The patient will take Atorvastatin 40 mg PO once daily at bedtime, and continue baseline Lisinopril 20 mg orally once daily. All prescriptions were transmitted electronically to the patient's pharmacy.

ACTIVITY & WOUND CARE PLAN

Keep dressing clean and dry. No heavy lifting greater than 10 pounds for two weeks. Surgical clinic follow-up scheduled in 10 days.